CIPRICIN – 500 TABLET

COMPOSITION
Each film coated tablet contains ciprofloxacin hydrochloride equivalent to ciprofloxacin 500mg.

DESCRIPTION:
A white to off white oblong shape film coated tablet with score line on one side.

ACTIONS:
Ciprofloxacin is a synthetic broad spectrum antibacterial agent, a fluoroquinolone, for intravenous and oral administration. It is effective against bacteria not only in the proliferation phase but also in the resting phase. Its bactericidal action is due to inhibition of the enzyme DNA gyrase which is needed for bacterial replication, thereby decreasing the ability of the bacteria to replicate. Resistance to ciprofloxacin develops slowly via multi-step mutation, and rapid one step resistance has not been observed.
Due to its different mode of action, ciprofloxacin does not cross-react with other antibiotics such as beta-lactams, aminoglycosides or macrolides, and organisms resistant to these antibiotics may be sensitive to ciprofloxacin. Ciprofloxacin is completely effective against β–lactamase-producing bacteria. Moreover, in vitro studies indicate that combination of ciprofloxacin with other antibiotics often lead to additive effects. Parallel resistance may occur with the gyrase inhibitors. Nevertheless, ciprofloxacin is often still effective against microorganisms which are resistant to other gyrase inhibitors.

Ciprofloxacin may be used in combination with the following antibiotics in the treatment of: *Pseudomonas*: azlocillin, caftazidime

Streptococci: mezlocillin, azlocillin, other effective β–lactam antibiotics

Staphylococci: β-lactam antibiotics, particularly isoxazolylpenicillins, vancomycin.

Anaerobes: metronidazole, clindamycin.

Ciprofloxacin is effective in vitro against most gram-negative organisms including *Pseudomonas aeroginosa*. It is also effective against gram-positive organisms such as *Streptococci and Staphylococci.*

However, anaerobes are generally less susceptible to ciprofloxacin.

Organisms which are generally regarded as sensitive to ciprofloxacin in *in vitro* studies include: *Escherichia coli, Klebsiella, Enterobacter Citrobacter, Salmonella, Shigella, Edwardsiella, Proteus, Providencia, Morganella, Serratia, Yersinia, Pseudomonas, Haemophilus, Neisseria,* *Moraxella (Branhamella), Campylobacter, Aeromonas, Vibrio, Listeria, Corynebacterium, Hafnia, Chlamydia.*

Brucella, Pasteurella, Legionella, Staphylococcus (including methicillin-susceptible and methicillin-resistant strains of *Staphylococcus aureus*).

Organisms which are moderately sensitive include *Gardnerella, Flavobacterium, Alcaligenes, Streptococcus algalactiae, Enterococcus faecalis, Streptococcus pyogenes, Streptococcus pneumoniae, Viridans group streptococci, Mycobacterium hominis, Mycobacterium tuberculosis and Mycobacterium fortuitum*.

Organisms resistant to ciprofloxacin include *Ureaplasma urealyticum*, most anaerobic bacteria including *Bacteroides fragilis* and *Clostridium difficile*, *Enterococcus faecium*, *Nocardia asteroides*. Moderately sensitive anaerobes include *Peptococcus*, *Peptostreptococcus*. Ciprofloxacin is ineffective against *Treponema pallidum*.

PHARMACOKINETICS

Ciprofloxacin is rapidly and well-absorbed after oral administration, with an absolute bioavailability of approximately 70%. Maximum serum concentrations are achieved after about 1 to 2 hours, and elimination half-life is about 4 hours. Ciprofloxacin is widely distributed in the body after oral administration, attaining high concentrations in the body fluids and tissues.

Approximately 40-50% of an orally administered dose is excreted unchanged in the urine, with smaller amounts being found in the faeces. Approximately 15% of an oral dose is recovered as metabolites in the urine. Food delays the time taken to achieve peak serum concentrations, but does not affect overall absorption.

In patients with impaired renal function, elimination half-life of ciprofloxacin is prolonged and dosage adjustment may be necessary. In contrast, no significant changes in ciprofloxacin pharmacokinetics have been observed in patients with stable chronic liver cirrhosis.

INDICATIONS

Consideration should be given to available official guidance on the appropriate use of antibacterial agents.

Uncomplicated and complicated infections caused by ciprofloxacin susceptible pathogens.

*Infections of the respiratory tract:

Ciprofloxacin can be regarded as an advisable treatment for pneumonias caused by
Klebsiella, Enterobacter, Proteus, E. coli, Pseudomonas, Haemophilus, Moraxella catarrhalis, Legionella and Staphylococcus.

*Infections of the middle ear (otitis media), of the paranasal sinuses (sinusitis), especially if these are caused by Gram-negative organisms including Pseudomonas aeruginosa or by staphylococci.

Infections of the eyes.

*Infections of the kidneys and/or the efferent urinary tract.

Infections of the genital organs, including adnexitis, gonorrhea, prostatitis and excluding vaginal infections.
Infections of the abdominal cavity (e.g. infections of the gastrointestinal tract or of the biliary tract, peritonitis).

Infections of the skin and soft tissue.

Infections of the bones and joints.

*Sepsis
Infections or imminent risk of infection (prophylaxis) in patients whose immune system has been weakened (e.g. patients on immunosuppressants or have neutropenia)

*Cipricin 500 should be only used:
• When Pseudomonas is considered AND the patient is allergic to antipseudomonal penicillins/ cephalosporin; or
• For resistant organism with no other alternative antibiotics available.

Prophylaxis of invasive infections due to Neisseria meningitidis.

<u>Children and adolescents</u>
Ciprofloxacin may be used in children for the second- and third-line treatment of complicated urinary tract infections and pyelonephritis due to Escherichia coli (age range applied in clinical studies: 1 – 17 years) and for the treatment of broncho-pulmonary infections of cystic fibrosis associated with Pseudomonas aeruginosa (age range applied in clinical studies: 5 – 17 years).

Treatment should only be initiated after careful benefit/risk evaluation, due to possible adverse events related to joints and/or surrounding tissues.

The clinical trials in children were performed in the indications listed above. For other indications clinical experience is limited.

Inhalational anthrax (post-exposure) in adults and in children

To reduce the incidence or progression of disease following exposure to aerosolized Bacillus anthracis.

DOSAGE AND ADMINISTRATION

Unless otherwise prescribed, the following guideline doses are recommended:

Indications	**Recommended dosages**
Respiratory tract infections (according to severity and organisms)	2 x 500 mg to 2 x 750mg
Urinary tract infections: • acute, uncomplicated • cystitis in pre-menopausal women • complicated	2 x 250 mg to 2 x 500 mg single dose 500 mg 2 x 500 mg to 2 x 750 mg
Genital infections - uncomplicated gonorrhea (including extragenital sites of infection) - adnexitis, prostatitis, epididymo-orchitis	1 x 500 mg 2 x 500 mg to 2 x 750 mg
Diarrhea	2 x 500 mg
Other infections (see Indications)	2 x 500 mg
Particularly severe, life threatening infections, i.e. -Recurrent infections in cystic fibrosis - Bone and joint infections - Septicemia - Peritonitis. In particular when Pseudomonas, Staphylococcus or Streptococcus is present	2 x 750 mg
Inhalational anthrax (post exposure)	2 x 500 mg
Prophylaxis of invasive infections due to Neisseria	1 x 500 mg as a single dose

Missed dose
If a dose is missed, it should be taken as anytime as but not later than 6 hours prior to the next scheduled dose. If less than 6 hours remain before the next dose, the missed dose should not be taken and treatment should be continued as prescribed with the next scheduled dose. Double doses should not be taken to compensate for a missed dose.

Geriatric patients (> 65 years)
Elderly patients should receive a dose as low as possible depending on the severity of their illness and the creatinine clearance. (see also 'Patients with renal and hepatic impairment')

Recommended doses for patients with renal impairment:

Creatinine Clearance [mL/min/1.73 m^2]	**Serum Creatinine[mg/100mL]**	**Total daily oral dose of ciprofloxacin**
30 to 60	1.4 to 1.9	maximum 1000 mg
Below 30	$\geq$2.0	maximum 500mg

Use in elderly patients:
The lowest possible effective dose should be used depending on their illness and creatinine clearance.

- Patients with renal impairment on hemodialysis

- For patients with creatinine clearance between 30 and 60 mL/min/1.73m^2 (moderate renal impairment) or serum creatinine concentration between 1.4 and 1.9 mg/100 mL, the maximum daily oral dose of ciprofloxacin should be 1000 mg
-For patients with creatinine clearance less than 30 mL/min/1.73m^2 (severe renal impairment) or serum creatinine concentration equal or higher than 2.0 mg/100 mL, the maximum daily oral dose of ciprofloxacin should be 500 mg.

- Patients with renal impairment on continuous ambulatory peritoneal dialysis (CAPD)

- The maximum daily oral dose of ciprofloxacin should be 1 x 500 mg Cipricin tablet.

- Patients with hepatic impairment

- In patients with impaired hepatic function no dose adjustment is required

- Patients with renal and hepatic impairment

- For patients with creatinine clearance between 30 and 60 mL/min/1.73m^2 (moderate renal impairment) or serum creatinine concentration between 1.4 and 1.9 mg/100 mL, the maximum daily oral dose of ciprofloxacin should be 1000 mg.
- For patients with creatinine clearance less than 30 mL/min/1.73m^2 (severe renal impairment) or serum creatinine concentration equal or higher than 2.0 mg/100 mL, the maximum daily oral dose of ciprofloxacin should be 500 mg .

- Children and adolescents

- Dosing in children with impaired renal and or hepatic function has not been studied.

Duration of treatment

The duration of treatment depends on the severity of the illness and on the clinical and bacteriological course. It is essential to continue therapy for at least 3 days after disappearance of the fever or of the clinical symptoms.

Mean duration of treatment

- 1 day for acute uncomplicated gonorrhea and cystitis
- up to 7 days for infections of the kidneys, urinary tract and abdominal cavity
- over the entire period of the neutropenic phase in patients with weakened body defenses
- a maximum of 2 months in osteomyelitis
- and 7 – 14 days in all other infections

In streptococcal infections, the treatment must last at least ten days because of the risk of late complications.

Infections caused by Chlamydia spp. should also be treated for a minimum of ten days.

Inhalational Anthrax (Post-exposure):
60 days from the confirmation of Bacillus anthracis exposure

Method of administration

Tablets are to be swallowed unchewed with fluid. They can be taken independent of mealtimes. If taken on an empty stomach, the active substance is absorbed more rapidly. Ciprofloxacin tablets should not be taken with dairy products (e.g. milk, yoghurt) or mineral-fortified fruit-juice (e.g. calcium-fortified orange juice) (See ‘Drug interaction’).

ROUTE OF ADMINISTRATION

Oral administration

CONTRAINDICATIONS

Ciprofloxacin is contraindicated in children or adolescents in the growing phase and in pregnant and nursing mothers as fluoroquinolones have been shown to cause arthropathy in immature animals. It is also contraindicated in those who have shown hypersensitivity reactions to ciprofloxacin or other quinolones.

Hypersensitivity to ciprofloxacin or other quinolone or any of the excipients

Concurrent administration of ciprofloxacin and tizanidine (see ‘Interaction with other medicinal products and other forms of interaction’).

ADVERSE EFFECTS
Reported side-effects of ciprofloxacin include:
Gastrointestinal: nausea, vomiting, abdominal pain/discomfort, dyspepsia, flatulence, anorexia, diarrhoea and rarely, oral candidiasis, dysphagia

Central nervous system: headache, restlessness, agitation, dizziness; rarely, nightmares, insomnia, hallucinations, tremors, convulsive seizures, lethargy, depression, sweating, psychotic reactions.

Musculoskeletal: joint or back pain, joint stiffness, gout flare-up

Cardiovascular: Rarely, fainting, palpitations, atrial flutter, hypertension, migraine. Very rarely, cardiopulmonary arrest, cerebral thrombosis, angina pectoris, myocardial infarction have been reported.

Skin: rashes, pruritis, urticaria, photosensitivity, flushing, erythema nodosum, very rarely – Stevens - Johnson Syndrome, petechiae and haemorrhagic bullae

Hypersensitivity: anaphylactic reactions (which may occur even after the first exposure), drug fever

Haematopoietic system: eosinophilia, leukocytopenia, agranulocytosis, haemolytic anaemia, thrombocytosis, altered prothrombin values

Renal: interstitial nephritis, polyuria, urinary retention, crystalluria, haematuria, renal calculi
Hepatic: increase in liver enzymes, cholestatic jaundice, hepatitis, hepatic necrosis

Special senses: impaired smell (parosmia), anosmia (reversible on discontinuation), blurred vision, tinnitus, bad taste.

Others: epistaxis, hemoptysis, dyspnea, bronchospasm, hyperglycaemia, increase in serum urea and creatinine

Exacerbation of myasthenia gravis
Post Marketing Experience

Musculoskeletal and connective tissue disorders*
Nervous system disorders*
General disorders and administrative site conditions*
Psychiatric disorders*
Eye disorders*
Ear and labyrinth disorders*

*Very rare cases of prolonged (up to months or years), disabling and potentially irreversible serious drug reactions affecting several, sometimes multiple, system organ classes and senses (including reactions such as tendinitis, tendon rupture, arthralgia, pain in extremities, gait disturbance, neuropathies associated with paraesthesia, depression, fatigue, memory impairment, sleep disorders, and impairment of hearing, vision, taste and smell) have been reported in association with the use of fluoroquinolones in some cases irrespective of pre-existing risk factors (see section Warnings and Precautions).

Psychiatric disorders

Rare: Depression (potentially culminating in suicidal ideations/ thoughts or suicide attempts and completed suicide)

Very Rare: Psychotic reactions (potentially culminating in suicidal ideations/ thoughts or suicide attempts and completed suicide)

WARNINGS AND PRECAUTIONS

Musculo-skeletal system: The risk of developing fluoroquinolone-associated tendonitis and tendon rupture is further increased in people older than 60, in those taking corticosteroid drugs, and in kidney, heart and lung transplant recipients. Patients experiencing pain, swelling, inflammation of a tendon or tendon rupture should be advised to stop taking their fluoroquinolone medication (Ciprofloxacin) and to contact their health care professional promptly about changing their antimicrobial therapy. Patients should also avoid exercise and using the affected area at the first sign of tendon pain, swelling or inflammation.

Tendonitis of the Achilles tendon may occur, which may lead to tendon rupture. If pain occurs in the Achilles tendon, the patient should be told to stop treatment and rest completely.

Safety of ciprofloxacin in children, adolescents, pregnant and lactating women has not been established (Please see Contraindication above). As ciprofloxacin may cause CNS stimulation, convulsions and increased intracranial pressure, it should be used with extreme caution in patients with history of epilepsy, CNS damage and severe cerebrovascular disease.

Alkalinization of the urine should be avoided during therapy with ciprofloxacin to prevent crystalluria. In addition, patients should be advised to drink fluids liberally.

Anaphylactic reactions have been reported with quinolones. Resuscitative measures should be readily available especially during parenteral administration.

As with other quinolones, ciprofloxacin may induce photosensitisation. It is therefore necessary for patients to avoid prolonged exposure to sunlight or ultraviolet light.

As with other antibacterial agents, prolonged use of ciprofloxacin may cause overgrowth of non-susceptible organisms and pseudomembranous colitis due to *Clostridium difficile*. If this occurs, the drug should be discontinued immediately and appropriate therapeutic measures initiated.

Exacerbation of myasthenia gravis

Fluoroquinolones have neuromuscular blocking activity and may exacerbate muscle weakness in person with myasthenia gravis. Post marketing serious adverse events, including deaths and requirement for ventilator support have been associated with flouroquinolones use in persons with myasthenia gravis. Avoid flouroquinolones in patients with known history of myasthenia gravis.

The use of Cipricin should be avoided in patients who have experienced serious adverse reactions in the past when using fluoroquinolones containing products (see section Adverse Effects/Undesirable Effects). Treatment of these patients with Cipricin should only be initiated in the absence of alternative treatment options and after careful benefit/risk assessment.

Aortic aneurysm and dissection

Epidemiologic studies report an increased risk of aortic aneurysm and dissection after intake of fluoroquinolones, particularly in the older population. Therefore, fluoroquinolones should only be used after careful benefit-risk assessment and after consideration of other therapeutic options in patients with positive family history of aneurysm disease, or in patients diagnosed with pre-existing aortic aneurysm and/or aortic dissection, or in presence of other risk factors or conditions predisposing for aortic aneurysm and dissection (e.g. Marfan syndrome, vascular Ehlers-Danlos syndrome, Takayasu arteritis, giant cell arteritis, Behcet's disease, hypertension, known atherosclerosis). In case of sudden abdominal, chest or back pain, patients should be advised to immediately consult a physician in an emergency department.

Prolonged, disabling and potentially irreversible serious adverse drug reactions

Very rare cases of prolonged (continuing months or years), disabling and potentially irreversible serious adverse drug reactions affecting different, sometimes multiple body systems (musculoskeletal, nervous, psychiatric and senses) have been reported in patients receiving fluoroquinolones irrespective of their age and pre-existing risk factors. Cipricin should be discontinued immediately at the first signs or symptoms of any serious adverse reaction and patients should be advised to contact their prescriber for advice.

Tendinitis and tendon rupture

Tendinitis and tendon rupture (especially but not limited to Achilles tendon), sometimes bilateral, may occur as early as within 48 hours of starting treatment with fluoroquinolones and have been reported to occur even up to several months after discontinuation of treatment. The risk of tendinitis and tendon rupture is increased in older patients (above 60 years of age), with renal impairment, patients with solid organ transplants, and those treated concurrently with corticosteroids*.
Therefore, concomitant use of corticosteroids should be avoided.

At the first sign of tendinitis (e.g. painful swelling, inflammation) the treatment with Cipricin should be discontinued and alternative treatment should be considered. The affected limb(s) should be appropriately treated (e.g. immobilisation). Corticosteroids should not be used if signs of tendinopathy occur.

*in patients receiving daily doses of 1000 mg levofloxacin.

Peripheral neuropathy

Cases of sensory or sensorimotor polyneuropathy resulting in paraesthesia, hypaesthesia, dysesthesia, or weakness have been reported in patients receiving quinolones and fluoroquinolones. Patients under treatment with Cipricin should be advised to inform their doctor and pharmacist prior to continuing treatment if symptoms of neuropathy such as pain, burning, tingling, numbness, or weakness develop in order to prevent the development of potentially irreversible condition (see section Adverse Effects/Undesirable Effects).

Psychiatric reactions

Psychiatric reactions may occur even after the first administration of fluoroquinolones, including Cipricin. In rare cases, depression or psychotic reactions can progress to suicidal ideations/thoughts and self-injurious behaviour, such as attempted or completed suicide (see section 'Undesirable effects'). In the event that the patient develops these reactions, Cipricin should be discontinued and appropriate measures instituted. Caution is recommended if Cipricin is to be used in psychotic patients or in patients with a history of psychiatric disease

PREGNANCY AND LACTATION

Pregnancy

The use of Cipricn-500 tablet is not recommended during pregnancy.

Lactation

Ciprofloxacin is excreted in breast milk. Due to the potential risk of articular damage, the use of Cipricn-500 tablet is not recommended during breast-feeding.

EFFECT ON ABILITY TO DRIVE

Due to its neurological effects, ciprofloxacin may affect reaction time. Thus, the ability to drive or to operate machinery may be impaired.

DRUG INTERACTIONS

Drugs known to prolong QT interval

Ciprofloxacin, like other fluoroquinolones, should be used with caution in patients receiving drugs known to prolong QT interval (e.g. Class IA and III antiarrhythmics, tricyclic antidepressants, macrolides, antipsychotics).

Chelation Complex Formation

The simultaneous administration of ciprofloxacin (oral) and multivalent cation-containing drugs and mineral supplements (e.g. calcium, magnesium, aluminium, iron), polymeric phosphate binders (e.g. sevelamer or lanthanum carbonate), sucralfate or antacids, and highly buffered drugs (e.g. didanosine tablets) containing magnesium, aluminium, or calcium reduces the absorption of ciprofloxacin. Consequently, ciprofloxacin should be administered either 1-2 hours before or at least 4 hours after these preparations. The restriction does not apply to antacids belonging to the class of H2 receptor blockers.

Food and Dairy Products

Dietary calcium as part of a meal does not significantly affect absorption. However, the concurrent administration of dairy products or mineral-fortified drinks alone (e.g. milk, yoghurt, calcium-fortified orange juice) with ciprofloxacin should be avoided because absorption of ciprofloxacin may be reduced.

Probenecid

Probenecid interferes with renal secretion of ciprofloxacin. Co-administration of probenecid and ciprofloxacin increases ciprofloxacin serum concentrations.

Metoclopramide

Metoclopramide accelerates the absorption of ciprofloxacin (oral) resulting in a shorter time to reach maximum plasma concentrations. No effect was seen on the bioavailability of ciprofloxacin.

Omeprazole

Concomitant administration of ciprofloxacin and omeprazole containing medicinal products results in a slight reduction of C_{max} and AUC of ciprofloxacin.

Effects of ciprofloxacin on other medicinal products

Agomelatine
In clinical studies, it was demonstrated that fluvoxamine, as a strong inhibitor of the CYP450 1A2 isoenzyme, markedly inhibits the metabolism of agomelatine resulting in a 60-fold increase of agomelatine exposure. Although no clinical data are available for a possible interaction with ciprofloxacin, a moderate inhibitor of CYP450 1A2, similar effects can be expected upon concomitant administration.

Tizanidine
Tizanidine must not be administered together with ciprofloxacin. In a clinical study with healthy subjects, there was an increase in serum tizanidine concentration (C_{max} increase: 7-fold, range: 4 to 21-fold; AUC increase: 10-fold, range: 6 to 24-fold) when given concomitantly with ciprofloxacin. Increased serum tizanidine concentration is associated with a potentiated hypotensive and sedative effect.

Methotrexate
Renal tubular transport of methotrexate may be inhibited by concomitant administration of ciprofloxacin, potentially leading to increased plasma levels of methotrexate and increased risk of methotrexate-associated toxic reactions. The concomitant use is not recommended.

Theophylline
Concurrent administration of ciprofloxacin and theophylline can cause an undesirable increase in serum theophylline concentration. This can lead to theophylline-induced side effects that may rarely be life threatening or fatal. During the combination, serum theophylline concentrations should be checked and the theophylline dose reduced as necessary.

Other xanthine derivatives
On concurrent administration of ciprofloxacin and caffeine or pentoxifylline (oxpentifylline), raised serum concentrations of these xanthine derivatives were reported.

Phenytoin
Simultaneous administration of ciprofloxacin and phenytoin may result in increased or reduced serum levels of phenytoin such that monitoring of drug levels is recommended.

Cyclosporin
A transient rise in the concentration of serum creatinine was observed when ciprofloxacin and cyclosporin containing medicinal products were administered simultaneously. Therefore, it is frequently (twice a week) necessary to control the serum creatinine concentrations in these patients.

Vitamin K antagonists
Simultaneous administration of ciprofloxacin with a vitamin K antagonist may augment its anti-coagulant effects. The risk may vary with the underlying infection, age and general status of the patient so that the contribution of ciprofloxacinto the increase in INR (international normalised ratio) is difficult to assess. The INR should be monitored frequently during and shortly after co-administration of ciprofloxacin with a vitamin K antagonist (e.g., warfarin, acenocoumarol, phenprocoumon, or fluindione).

Duloxetine
In clinical studies, it was demonstrated that concomitant use of duloxetine with strong inhibitors of the CYP450 1A2 isozyme such as fluvoxamine, may result in an increase of AUC and C_{max} of duloxetine. Although no clinical data are available on a possible interaction with ciprofloxacin, similar effects can be expected upon concomitant administration.

Ropinirole
It was shown in a clinical study that concomitant use of ropinirole with ciprofloxacin, a moderate inhibitor of the CYP450 1A2 isozyme, results in an increase of C_{max} and AUC of ropinirole by 60% and 84%, respectively. Monitoring of ropinirole-related side effects and dose adjustment as appropriate is recommended during and shortly after co-administration with ciprofloxacin.

Lidocaine
It was demonstrated in healthy subjects that concomitant use of lidocaine containing medicinal products with ciprofloxacin, a moderate inhibitor of CYP450 1A2 isozyme, reduces clearance of intravenous lidocaine by 22%. Although lidocaine treatment was well tolerated, a possible interaction with ciprofloxacin associated with side effects may occur upon concomitant administration.

Clozapine
Following concomitant administration of 250 mg ciprofloxacin with clozapine for 7 days, serum concentrations of clozapine and N-desmethylclozapine were increased by 29% and 31%, respectively. Clinical surveillance and appropriate adjustment of clozapine dosage during and shortly after coadministration with ciprofloxacin are advised.

Sildenafil
C_{max} and AUC of sildenafil were increased approximately twofold in healthy subjects after an oral dose of 50 mg given concomitantly with 500 mg ciprofloxacin. Therefore, caution should be used prescribing ciprofloxacin concomitantly with sildenafil taking into consideration the risks and the benefits.

Zolpidem
Co-administration of ciprofloxacin may increase blood levels of zolpidem, concurrent use is not recommended.

OVERDOSAGE AND TREATMENT

If overdosage occurs, the unabsorbed drug should be removed by induction of emesis or gastric lavage. The patient should be well-hydrated and treated symptomatically. Other quinolones have occasionally caused convulsions, but no prophylactic anticonvulsant therapy is required. Reversible renal toxicity due to acute, excessive oral overdosage has been reported and patients should be monitored for renal function accordingly.
Apart from routine emergency measures, it is recommended to administer magnesium or calcium-containing antacids to help reduce the absorption of ciprofloxacin.
Only a small amount of ciprofloxacin (<10%) is removed from the body by haemodialysis or peritoneal dialysis.

STORAGE
Store in a dry place below 30°C.
Protect from light.
Keep out of reach of children.

PRESENTATION
Blister of 10 tablets, box of 10, 25 or 50 strips.

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